भारत सरकार

Government of India

आधार

दीपक अग्निहोत्री

Deepak Agnihotri

जन्म तिथि / DOB : 27/09/1997

पुरुष / Male

आधार पहचान का प्रमाण है, नागरिकता का नहीं।
Aadhaar is a proof of identity, not of citizenship.

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मेरा आधार, मेरी पहचान

Issue Date: 22/02/2015

OPD Sheet

Regn. No.	: OP25-26-133087	Visit Date	: 13-08-2025 08:28 AM
Patient Name	: MR. DEEPAK AGNIHOTRI	Age / Gender	: 27 Yr / M
MRN	: SR1042892	Mobile No.	: 8448870839
Category	: Checkmed Private Limited	Department	: INTERNAL MEDICINE
Doctor	: Dr. MADHURIMA GUPTA	Referred By	:
Address	: Faridabad Haryana	Token No.	: IM_S19586_3

C/o Routine checkup
Constipation, Gaseous distention

Family H/o - CAD ischemic DM II (father - Post PTCA + stenting)
DM II (Mother)
Surgical H/o -
Past H/o - } nil
Drug Allergy -

H/o Smoking H/o Alcohol once/wk

GC Conscious Afebrile Chest clear
BP 120/70 mg Pulse 84/min Regular

Observation

- Echo - Bicuspid AV c̄ mild AR

Advice

- Refer to Cardiologist

- Dyslipidemia
LDL + 146 mg/dl
- Grade I fatty liver

Dr. Madhurima Gupta
Senior Consultant - PHC
MBBS, DGO, PGDFW, PGDHM
Registration No: HMC 10090
Sarvodaya Hospital & Research Centre

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sarvodaya HEALTHCARE

MC-2277

SARVODAYA HOSPITAL
SECTOR-8, FARIDABAD | HELPLINE: 1800 313 1414

MR. DEEPAK AGNIHOTRI [SR1042892]

Age	: 27 Yr / Male	Collection on	: 13-08-2025 08:36 AM | SHAYB224,0183000,SHAYB223
Reg ID	: OP25-26-133087 | Order # 666406	Accepted on	: 13-08-2025 08:36 AM
Req by	: Dr. MADHURIMA GUPTA	Report Date	: 13-08-2025 12:05 PM
Address	: Faridabad, Faridabad, Haryana, India, 121002	Status	: Finalized

CLINICAL BIOCHEMISTRY

INVESTIGATION	RESULT		UNIT	BIOLOGICAL REFERENCE RANGE
Lipid Profile				
S. Cholesterol	263	H	mg/dl	0 - 200
S. Triglycerides	113		mg/dl	0 - 150
S. HDL Cholesterol	44.0		mg/dl	35 - 55
VLDL Cholesterol	22.6		mg/dl	5 - 32
S. LDL Cholesterol	196.4	H	mg/dl	0 - 100
TOTAL / HDL Cholesterol Ratio	5.98	H		0 - 5
LDLC/HDLC Ratio	4.46	H		0 - 3.55
Non-HDL Cholesterol	219	H	mg/dl	0 - 160

Interpretation: :

TRIGLYCERIDES level>250mg/dl is associated with an approximately 2-fold greater risk of coronary vascular disease.Elevation of triglyceride can be with obesity,medication,fast less than 12hrs. alcohol intake,diabetes melitus and pancreatitis. **CHOLESTROL**, its is fraction and triglyceride are important plasma lipids indefining cardiovascular disease.Highest acceptable and optimum values of cholestrol,value of cholestrol vary with age.value above 220 mg/dl are associated with increased risk of chd regardless of HDL & LDL values. **HDL-CHOLESTROL** level < 35 mg/dl is associated with increased risk of coronary disease even in the face of desirable level of cholesterol LDl cholestrol.

Test Method :- Cholesterol-Enzymatic Colorimetric with cholesterol Easterase/Triglycerides-Enzymatic colorimetric (Glycerol Phosphate oxidase)/HDL-Homogeneous Enzymatic / LDL-Calculated/,VLDL Calculated.

Sample Type:- Serum

FBS (Blood Sugar Fasting)	109	mg/dl	74 - 109

nterpretation: :

lood glucose may be elevated in diabetes mellitus, gestational diabetes acute hepatitis, acute pancreatitis and Addison s iseases .

ypoglycaemia is associated with neonatal respiratory distress syndrome,toxaemia of pregnancy, congenital enzyme defects eye s syndrome ,alcohol ingestion, hepatic dysfunction, insulin-producing pancreatic tumours, insulin antibodies, nonpancreat oplasms, septicaemia and chronic renal failure.

st Method:- Hexokinase

mple Type : Fluoride Plasma

RFT III			
Blood Urea	21.1	mg/dl	16.6 - 48.5
Serum Creatinine	0.81	mg/dl	0.7 - 1.2
Sodium(Na+)	136.4	mmol/L	136 - 145

Potassium(K+)	4.13	mmol/L	3.5 - 5.1
Chloride(CL-)	99.3	mmol/L	98 - 107
Uric Acid	**7.15**	H mg/dl	3.4 - 7
Calcium(Total)	**10.3**	H mg/dl	8.6 - 10
S.Phosphorus	2.88	mg/dl	2.5 - 4.5

Interpretation: :

Urea: Levels may be elevated due to renal causes,Acute glomerulonephritis,chronic nephritis,polycystic kidney,tubular necrosis,and nephrosclerosis,obstruction of the urinary tract,acute renal failure,chronic renal failure and prerenal azotaemia.

Creatinine: Measurements of creatinine is useful in diagnosis and treatment of renal disease and is useful in the evaluation of kidney glomerular function and in monitoring renal dialysis.

Electrolytes: Affects most metabolic processes. They serve top maintain osmotic pressure and hydration of various body fluid compartments, proper body pH, and regulation of appropriate heart and muscle functions. Electrolytes are also involved in oxidation reduction reactions and participate as essential parts or, co factors, in enzyme reactions.

Calcium: A significant reduction in calcium ion concentration result in muscle tetany.A higher than normal concentration of calcium ions produces lowered neuromuscular excitability and muscle weakness along with other more complex symptoms.

S.Phosphorous Hypophosphataemia :Decreased intake or absorption of phosphate such as occurs in Vit D deficiency ,malabsorption , use of oral phosphate binders and primary PTH excess, increased excretion such as occurs in secondary PTH excess, post renal transplant and re-feeding

starved patients and from redistribution of phosphate eg : hyperalimentation , recovery from diabetic ketoacidosis and respitatory alkalosis.

Hyperphosphataemia : Increased intake such as occurs in intravenous therapy and phosphate enemas reduced excretion such as occurs in acute and chronicrenal failure, low PTH or resistance to PTH and Vitamin D toxicity and redistribution of phosphate that occurs in tumour lysis, rhabdomyolysis and heat stroke.**Uric acid** is the major product of purine catabolism in humans.

Hyperuricaemia : Primary hyperuricaemia is associated with gout, Lesch -Nyhan syndrome, Kelley Seegmiller syndrome and increas phosphoribosyl pyrophosphate synthase activity. Secondary hyperuricaemia is seen in increased nutritional purine uptake increased uric acid excretion in the urine. Renal insufficiency ,myeloprolliferative diseases ,haemolytic diseases, psoriasis, polycythemia vera,type glycogen storage

Test Method :-Urea-Kinetic With Glutamatedehydrogenase(GLDH)/,Creat-Jaffe ,Sodium,Potassium and Chloride -ISE Indirect. Calcium-NM-BAPTA,S.Phosphorous-Molybdate UV, Uric acid-URICASE

Sample Type :- Serum

LFT III

SERUM BILIRUBIN			
Total Bilirubin	0.54	mg/dl	0 - 1.2
Direct Bilirubin	0.26	mg/dl	0 - 0.3
Indirect Bilirubin	0.28	mg/dl	0 - 0.6
SGPT(ALT)	24.8	u/L	0 - 50
SGOT(AST)	23.4	u/L	0 - 50
Alkaline Phosphate(ALP)	**140**	H u/L	40 - 129
Total Protein	8.02	g/dL	6.6 - 8.7
Albumin	4.69	g/dL	3.5 - 5.2
A/G Ratio	1.41	g/dL	0.9 - 2
GAMMA GT	34.1	u/L	10 - 71
Globulin	3.33	gm/dl	2 - 3.5

terpretation: :

Liver functions tests are interpreted keeping in view all the individual tests included.The evaluation of AST activity in relation to is a useful indicator of liver damage. Ratios <1.0 are indicative of mild liver damage and are particularly associated with diseases an inflammatory nature. Ratios >1.0 are indicative of severe liver diseases usually involving necrosis. Alkaline phosphatase is usually seen in the scenario of post hepatic lesions. Total protein and albumin being a measure of the amino acid metabolism in t liver .And T. BIL and D. Bil together being a measure of the excretory function of the liver,GGTP is a sensitive indicator of biliary tr disease.

Test Method:-Bil T-Colorimetric Diazo/BIL D - Diazo /Bil ID - Calculated /ALT and AST- IFCC With P-5-P/AlkalinePhosphatase-IFCC/Protein-Biurate/Albumin-Bromocresol Green/GGT:L-GAMA-GLUTAMYL-3-CARBOXY-4-NITROANILIDE SUBSTRATE,IFCC/Globulin & A/G ratio - Calculated.

Sample Type :- Serum

CRP (C-Reactive Protein)	4.50	mg/L	0 - 5

Interpretation: :
Interpretation:-CRP is an acute phase reactant which is used in inflammatory disorder for monitoring course and effect of therapy it is most use full as an indicator in tissue injury and infection .
Test Method :-Immunoturbidimetry
Sample Type :- Serum

CLINICAL PATHOLOGY

INVESTIGATION	RESULT	UNIT	BIOLOGICAL REFERENCE RANGE
Urine R and M			
Physical Examination			
Color	Yellow		Pale yellow
Transparency	Clear		Clear
Chemical Examination			
pH	6.0		5 - 8
Specific Gravity	1.015		1.010 - 1.035
Albumin	Negative		Negative
Sugar	Negative		Negative
Blood	Negative		Negative
Ketone	Negative		Negative
Nitrite	Negative		Negative
Bilirubin	Negative		Negative
Urobilinogen	Normal		Normal
Microscopic Examination			
Pus cells	1-2	/hpf	0 - 2
RBCs	00	/hpf	0 - 5
Epithelial cells	0-1	/hpf	Nil
Cast	NIL		Nil
Crystal	NIL		

Others		
Bacteria		
Yeast cells	NIL	
	NIL	Nil

Interpretation: :
Speciman Type :- Urine
Method :- Manual strip test/ Microscopy

HAEMATOLOGY

INVESTIGATION	RESULT	UNIT	BIOLOGICAL REFERENCE RANGE
HbA1C (Glycosylated Hb)	5.4	%	4 - 5.6

Interpretation: :

Non diabetic adults: 4 -5.6 ; At risk (Prediabetes) :5.7-6.4 ; Diabetic >= 6.5

In vitro quantitative determination of HbA1C in whole blood is utilized in long term monitoring of glycemia. The HbA1C level correlates with the mean glucose concentration prevailing in the course of the patient recent history (approx - 6-8 weeks)and therefore provides much accurate determinations of blood glucose or urinary glucose.(Note : Average Blood Glucose value is calculated from HbA1C value done and it indicates average blood sugar level over past three months.)

ADA criteria for correlation betweenHbA1c and Mean plasma glucose

HbA1c (%) -- Mean plasma glucose (mg/dl)

6.0 -- 126

7.0 -- 154

8.0 -- 183
9.0 -- 212
10.0 -- 240
11.0 -- 269
12.0 – 298

CBC With ESR			
Hb(Haemoglobin)	16.1	gm/dl	13 - 17
HCT	48.5	%	40 - 50
RBC	**5.71**	H 10^6/uL	4.5 - 5.5
MCV	84.9	fL	83 - 101
MCH	28.1	pg	27 - 32
MCHC	33.2	g/dL	31.5 - 34.5
RDW	13.7	%	11.6 - 14
TLC(Total Leucocyte count)	7.45	thou/mm3	4 - 10
DLC(Differential Leucocyte Count)			
Neutrophils	67.3	%	40 - 80
Lymphocyte	23.5	%	20 - 40

Monocyte	6.5	%	2 - 10
Eosinophils	2.4	%	1 - 6
Basophils	0.3	%	0 - 2
Absolute Neutrophils count	5.01	thou/mm3	2 - 7
Absolute Lymphocytes Count	1.75	thou/mm3	1 - 3
Absolute Monocytes Count	0.48	thou/mm3	0.2 - 1
Absolute Eosinophils Count	0.18	thou/mm3	0.02 - 0.5
Absolute Basophils Count	0.02	thou/mm3	0.02 - 0.1
Platelets	258	10^3/cmm	150 - 410
ESR(Erythrocyte Sedimentation Rate)	8	mm/hr	0 - 15

Interpretation: : Test Method :-WBC :- SF CUBE Cell Analysis,Hb :- Colorimetric,RBC/PLT :- Electrical Impedance MCV/HCT/MCH/MCHC/RDW-CV/Absolute WBC counts :- Calculated,ESR: Modified Westergren.
Sample Type: EDTA Whole Blood
* Critical value - Please correlate clinically

HORMONES

INVESTIGATION	RESULT	UNIT	BIOLOGICAL REFERENCE RANGE
Thyroid Profile (Total)			
Total T3	1.39	ng/ml	0.8 - 2
Total T4	6.46	ug/dl	5.1 - 14.1
TSH (Thyroid Stimulating Hormone)	1.880	uIU/mL	0.27 - 4.2

Interpretation: :
Triodothyronine(T3):-Serum measurements are used for diagnosing disorders of thyroid function as well as the following course o therapy for hyperthyroidism.**Thyroxine(T4):-**Serum T4 measurement are useful only in conjunction with other thyroid function tests such as TSH ,FT4 Total T3 as Thyroxine binding globulin (TBG) capacity has a pronounced effect on the concentration of T4.
TSH : Stimulates the Production and Secretion of the Metabolically Active Thyroid Hormones, Thyroxine (T4) and Triiodothyronine (T3). The assay is used as an aid in the Assessment of Thyroid Status, Diagnosis of Thyroid Disease, and Treatment of Thyroid Disease.

Test Method :- ELECTROCHEMILUMINESCENCE IMMUNOASSAY.
Sample Type :- Serum

SEROLOGY

INVESTIGATION	RESULT	UNIT	BIOLOGICAL REFERENCE RANGE
VDRL (RPR)			
VDRL (RPR)	Non-Reactive		Non-Reactive

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SARVODAYA HOSPITAL
SECTOR-8, FARIDABAD | HELPLINE: 1800 313 1414

PREPARED AND CHECKED BY

Dr. Ranjan Kuma
MBBS, MD (Pathology
Registration No-DMC – 3254

Above Results are of the Tests performed at Sarvodaya Hospital and Research Centre Lab adhering to Quality testing Guidelines.

MR. DEEPAK AGNIHOTRI [SR1042892]

27 Yr | Male | Ph : 8448870839

Req by : Dr. MADHURIMA GUPTA

Reg ID : OP25-26-133087 | **Token** : CL17

Request : 13-08-2025 08:35 AM | **Reported :** 13-08-2025 11:41 AM | **Status** : Finalized

Address : Faridabad, Faridabad, Haryana, India, 121002

Observation :

2D ECHO

DEPARTMENT OF CARDIOLOGY

M-Mode Dimension		Normal
Aortic Root Diameter	24mm	20-35mm
Left Atrial Dimension	32mm	19-40mm
Left Ventricular ED Dimension	44mm	35-56mm
Left Ventricular ES Dimension	25mm	22-40mm
Inter Ventricular Septal Thickness (ED)	10mm	06-12mm
Inter Ventricular Septal Thickness (ES)	14mm	
Left Ventricular (LVPW) D	10mm	02-10mm
Left Ventricular (LVPW) S	14mm	
LV Ejection - Fraction	55%	55-75 %

DOPPLER STUDY

Mitral Valve :- Trace MR

Mitral Valve E Velocity	85	cm/sec
Mitral Valve A Velocity	62	cm/sec

Aortic Valve :- Bicuspid AV with Trace AR

AV Velocity	177	cm/sec
AV Gradient	12.0	mmHg

Tricuspid Valve :- Trace TR

TR Velocity		cm/sec
PASP	29	mmHg

Pulmonary Valve :- Normal

PV Velocity	104	cm/sec
PV Gradient	4.0	mmHg
PAEDP		mmHg

Clot/ Vegetation	Nil
Pericardial Effusion	Nil

Regional Wall Motion Abnormality	**Nil**

Findings:-

Normal LV systolic function. No RWMA; LVEF 55%

Normal cardiac chamber dimension.

Normal RA/RV size with normal RV systolic function.

Trace MR.

Bicuspid AV with Trace AR

Trace TR [PASP - 29 mmHg].

Normal mitral inflow pattern .

No intra cardiac clot / vegetation seen.

IVC euvolemic (approx size 1.2cm) with more than 50% respiratory collapsibility.

Impression :

No RWMA with LVEF 55%
Normal mitral inflow pattern .

Ms. SHUCHI THUKRAL

for Dr Nitasha Singh

Dr. NITASHA SINGH
DNB (Medicine)
Associate Director & HoD

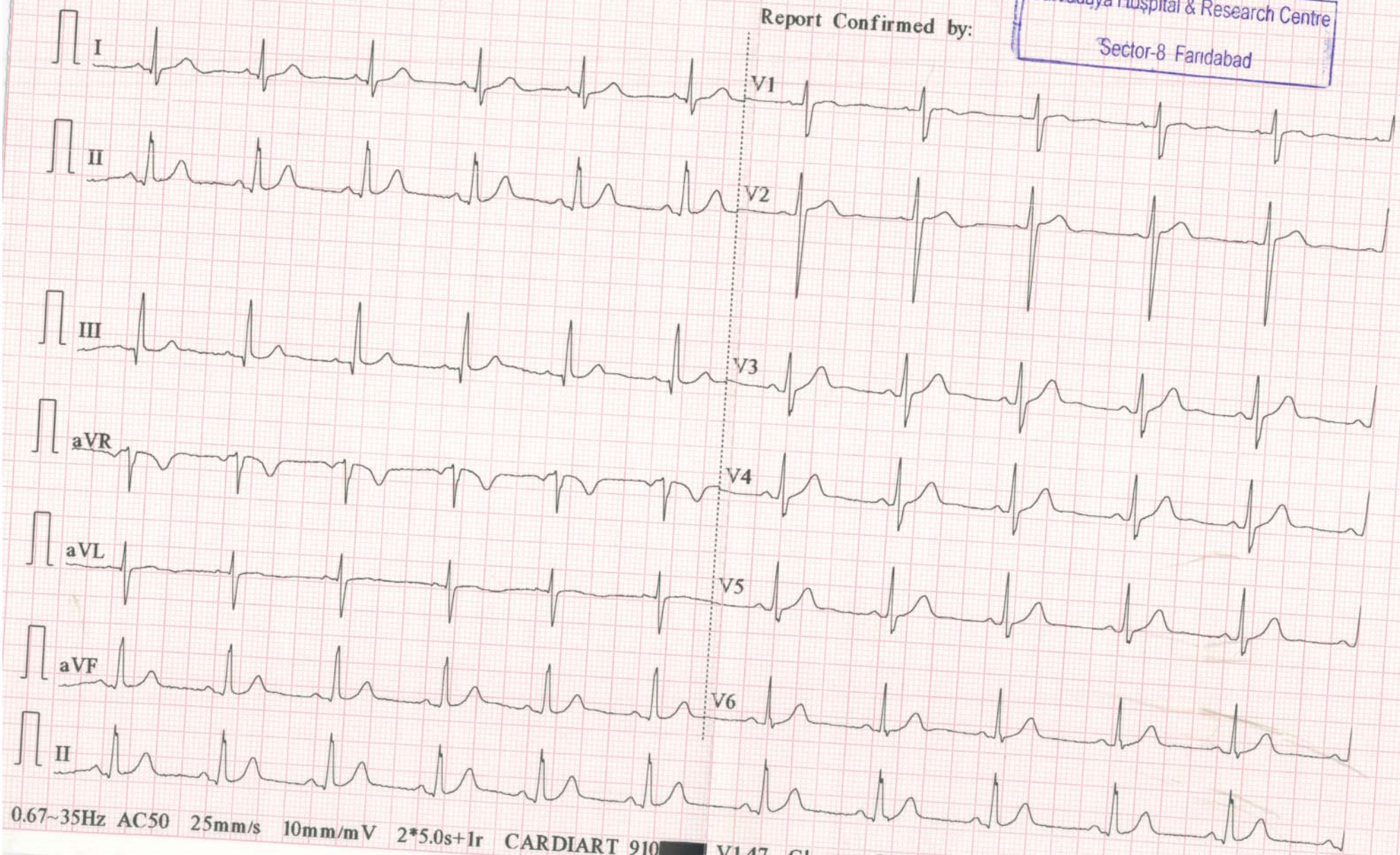
Mr deepak agnihotri
Male 27Years
Req. No. :

HR	: 69	bpm
P	: 98	ms
PR	: 132	ms
QRS	: 96	ms
QT/QTcBz	: 386/414	ms
P/QRS/T	: 49/75/58	°
RV5/SV1	: 0.981/0.591	mV

Diagnosis Information:
Sinus rhythm
Normal ECG

Report Confirmed by:

Sarvodaya Hospital & Research Centre
Sector-8 Faridabad



0.67~35Hz AC50 25mm/s 10mm/mV 2*5.0s+1r CARDIART 910[redacted] V1.47 Glasgow V28.6.7 Sarvodaya Hospital Faridabad

MR. DEEPAK AGNIHOTRI [SR1042892]

27 Yr | Male | Ph : 8448870839

Req by : Dr. MADHURIMA GUPTA

Reg ID : OP25-26-133087 | **Token** : CL23

Request : 13-08-2025 08:35 AM | **Reported :** 13-08-2025 12:07 PM | **Status** : Finalized

Address : Faridabad, Faridabad, Haryana, India, 121002

Observation :

TMT (Treadmill Test)

Protocol	**Bruce**	**MPHR**	**193 bpm**
Duration of exercise	**09 Min.27 Sec**	**90% of MPHR**	**173 bpm**
Reasons for termination	**THR Achieved**	**Peak HR Achieved**	**166 bpm**
Blood Pressure (mmHg)	**Baseline BP: 120/70**	**% Target HR**	**86%**
	Peak BP: 150/80	**METS**	**13.50**

Results:-

Had excellent effort tolerance.
Appropriate Heart Rate Response.
Appropriate B.P. Response.
No significant ST depression during the test.
No significant arrhythmia during the test.
No S3/S4 Gallop during the test.

Impression :

»The exercise stress test is "Negative" for Provocable Myocardial ischemia. PLEASE CORRELATE CLINICALLY.

Ms. SHUCHI THUKRAL

Nitasha

Dr. NITASHA SINGH
DNB (Medicine)
Associate Director & HoD

MR. DEEPAK AGNIHOTRI [SR1042892]

27 Yr | Male | Ph : 8448870839	
Req by : Dr. MADHURIMA GUPTA	**Token** : UL22826
Reg ID : OP25-26-133087	**Status** : Finalized
Request : 13-08-2025 08:35 AM | **Reported** : 13-08-2025 10:08 AM	
Address : Faridabad, Faridabad, Haryana, India, 121002	

Ultrasound Whole Abdomen

Observation :

Technique: Longitudinal and transverse images through the abdomen were performed utilizing trans abdominal approach as ordered. (Model no. 279640SU7).

Findings:
Liver: is enlarged in size (16.3 cm) and shows mild fatty infiltration. Hepatic biliary system & porta hepatis appear normal.
Gall Bladder: is normally distended. Wall thickness is normal. No calculus/ mass seen. CBD is normal.
Pancreas: is normal in size & echotexture.
Right kidney: is normal in shape, size, echotexture & position. Pelvicalyceal system is normal. No calculus seen. No mass seen.
Left kidney: is normal in shape, size, echotexture & position. Pelvicalyceal system is normal. No calculus seen. No mass seen.
Spleen: is normal in size & echotexture.
Urinary Bladder: is normally distended. Wall thickness within normal limits. No calculus/ mass seen.
Prostate: is normal in size & echotexture.
No free fluid is seen.
Generalized gaseous distension of colonic loops are seen.

Impression :

- **Hepatomegaly with grade I fatty liver.**
- **Generalized gaseous distension of colonic loops.**

Adv : Clinical correlation.

Vijaykumar

Mr. SAMRAT SAHA

Dr. VIJAY KUMAR GOYAL
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